15.6 Placental Abruption

Assessment	Nursing Diagnosis	Plan / Expected Outcome	Nursing Interventions	Evaluation
Subjective • "Severe constant pain in my abdomen." **Objective** • Uterus hard and tender • Dark vaginal bleeding • Pain, 9/10	**ACTUAL** **Acute pain** related to bleeding behind the placenta as evidenced by constant abdominal pain and a hard uterus.	Pain controlled.	• Give analgesia as prescribed. • Get her ready for urgent delivery. • Watch the tone of the uterus.	Controlled: **goal met**.
Subjective • *No specific complaint* **Objective** • BP 90/60 mmHg, HR 120/min • May be bleeding behind the placenta (concealed)	**ACTUAL** **Deficient fluid volume** related to hemorrhage as evidenced by hypotension and tachycardia.	Vital signs stable and urine output adequate.	• Put in two large-bore IV cannulas. • Give fluids and blood as prescribed. • Check clotting for DIC. • Record urine output every hour.	Stable: **goal met**.
Subjective • *No specific complaint* **Objective** • Fetal heart pattern abnormal	**RISK** **Risk for disturbed maternal-fetal dyad** as evidenced by risk factors: placental separation.	Baby delivered safely with no delay.	• Continuous fetal heart monitoring. • Call the obstetric and neonatal teams straight away. • Get her ready for urgent cesarean birth.	Safe delivery: **goal met**.